

**Analyzing the Healthcare Infrastructure in Kosovo from Gender Perspective:
Access, Education and Services**

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Abstract

This research investigates the healthcare infrastructure for Kosovar citizens, with a particular focus on women's healthcare. It analyzes access to both public and private medical services, the legal framework that governs healthcare, and the range of factors that influence a woman's ability, or inability to access necessary care. When referring to "access" it should be noted that our research addresses the term in its broadest sense, encompassing all elements that assist or prevent recipients of medical services. These would include, but are not limited to, geographic living situation, socio-economic status, education background, and family obligations. The central question of this research is: *How do gender dynamics and geographic disparities affect access to and quality of healthcare for women in Kosovo?* Through background research, interviews with various stakeholders, and surveys from patients and healthcare providers, our findings outline the strengths and weaknesses of the established health system in Kosovo. Proposed recommendations to public and private institutions are given at the end of the article to further promote positive and necessary change to the community.

Introduction

A healthcare system is not complete, equal, or democratic until it cares for the women under its purview. Healthcare has been enshrined as a human right by many international institutions: the World Health Organization has said healthcare is indivisible from other human rights, and has repeatedly emphasized the role of the state to deliver quality healthcare.¹ This involves several important elements, including well designed healthcare laws, efficient hospital systems, empowering healthcare education, and correct implementation of the law into practice.

Since 2004, Kosovo has been working to create a legal framework surrounding healthcare that claims to be “equal,” “easy to access,” and “equitable”.² Enshrining these values in the legal code is a great step forward. However, various reports and interviews over the years from both national and international publications have shed light on the disparities that still remain in the healthcare system today. Various NGOs have begun work with the mission of increasing access to the healthcare system; women of various backgrounds have come forward with their subpar experiences; and it is clear there are gaps to fill within this system. These gaps primarily affect women at the margins - women who are of lower economic status, have less access to education, live in rural areas or are a part of marginalized communities. There are various possible explanations for this, including a lack of female providers, remaining stigmas about women’s health issues such as breast cancer, and inadequate provision of care in geographically remote areas. This study aims to investigate these possibilities.

Improving access to healthcare is not just about the medical sector; research has shown that equal access to healthcare promotes economic and social equality as well. When women are able to live healthy lives, they are able to prosper, benefiting Kosovo and the world at large. The World Economic Forum and the McKinsey Health Institute have found that “reducing the women’s health gap improves lives and can potentially unlock a \$ 1 trillion GDP opportunity annually by 2040.”³ While improving women’s healthcare in Kosovo could be costly, it is an investment with near guaranteed returns.

¹World Health Organization. *Human Rights and Health*. 13 Dec. 2017, <https://www.who.int/news-room/fact-sheets/detail/human-rights-and-health>.

²Republic of Kosovo. *Law No. 04/L-125 on Health*. Ministry of Health, 2012, https://cps.rks-gov.net/wp-content/uploads/2020/08/LAW_No_04_L-125_ON_HEALTH.pdf.

³Pérez, Lucy. "Ushering in a New Era of Investment for the Health of Women." *World Economic Forum*, 17 Jan. 2024, <https://www.weforum.org/stories/2024/01/ushering-in-a-new-era-of-investment-for-the-health-of-women/>.

Thus, this research undertakes a comprehensive examination of the status of women's healthcare in Kosovo using primarily qualitative methods to suggest short and long term policy changes that would benefit the country's women, society, and democracy. By contributing to an ongoing effort by local and international institutions, this research aims to empower policy-makers and stakeholders to create positive change and progress toward a more equitable healthcare system in Kosovo.

Methodology

This research utilizes a mixed-methods approach with both qualitative and quantitative data. This methodology allows for an overview of the current status of women's healthcare in Kosovo through the perspectives of key institutional stakeholders, everyday Kosovar women, civil society organizations, and other reliable data sources.

Qualitative Data Collection

Qualitative data collection for this study was implemented via semi-structured interviews with relevant professionals who have significant experience in women's healthcare issues and their promotion in Kosovo. Interviews were conducted with:

- Dr. Mujko-Nimani, Head of Office at UNFPA Kosovo
- Ms. Nafije Latifi, Director of Jeta-Vita
- Ms. Hana Kurti, PR Officer for Action for Mothers and Children
- PRAK - Patients Right Association In Kosovo

Quantitative Data Collection

Two surveys were conducted to complement the quantitative data:

- Surveys were conducted using Google Forms and included responses from 56 women across Kosovo. The questions focused on the accessibility of the healthcare system and covered topics such as the frequency of doctor visits, areas in need of improvement within medical services, and the regularity of preventative screenings for diseases that specifically affect women.

- A survey of healthcare professionals was also conducted using Google Forms. This data gathered 6 responses, primarily from providers in Pristina. The responses came from the perspective of healthcare workers and their understanding of healthcare accessibility and affordability for their patients.

Document Review and Analysis

This research also includes an extensive review of legal frameworks and institutional reports surrounding healthcare delivery and infrastructure. The laws about healthcare and health insurance are reviewed. Additionally, a literature review of existing, peer-reviewed journal articles is included, and a thematic analysis was conducted to determine the key ideas throughout already existing research.

Literature Review

The literature review included in this study consists of a general overview from institutional publications, and then a thematic review of three key topics: patient outcomes, hospital management, and workforce conditions and practices. From this, it is possible to draw conclusions about what data is generally available as well as key trends for further discussion.

Institutional Report Review

UN Women recently published their Kosovo Country Profile, a general report on the status of women's rights in the country. It covers a variety of topics, from safety to education to healthcare, which is the section to take note of here. The report makes note of the fact that sexual education is not a required element in public education. In part due to this, only 40% of youth use contraception, which is substantially lower than the European average of 70%.⁴ There are no comprehensive statistics available about abortion rates. As for preventative care, the report notes

⁴ UN Women. "Kosovo Gender Country Profile." UN, 2024, https://www.eeas.europa.eu/sites/default/files/documents/2024/UNWOMEN_GEF_ENG.pdf, 35.

that most women do not undergo PAP examinations, breast ultrasounds, or mammograms⁵. The report notes a general lack of awareness in regards to HIV and STIs. Overall, the report highlighted a massive information provision and knowledge gap in Kosovo about all aspects of reproductive health.

A study conducted by Artpolis and supported by the Austrian Development Cooperation conducted an analysis of induced abortions and women's reproductive health from 1999 to 2019 in Kosovo. This study noted that there is no existing record of exact data about abortions in Kosovo, similarly to the previous report. The report explained that the Kosovo Obstetrics and Gynecology Association is concerned about the quality and safety of abortions conducted at private clinics.⁶ Additionally, there is discussion about concerns related to usage of abortion-induction pills without doctor's consultation or prescription, as well as ongoing concerns related to gender-biased sex selection⁷. Overall, the study called for better data reporting, improved sex education, and increasing conversations around family planning and contraception in healthcare environments.

A third major overview report was published in 2016 by the Rockefeller Center at Dartmouth College, who conducted a nation-wide survey about healthcare issues in Kosovo. The researchers surveyed over 1000 participants across Kosovo on their knowledge of the healthcare system, its accessibility, and their patient rights. Based on their survey, the researchers identified several gaps when it came to healthcare access, particularly the need to increase care for rural populations and knowledge about primary and preventative care.⁸

Generally, these reports demonstrate several important takeaways. First, they all reflect a general improvement in the healthcare system following the war. However, all of the reports note various areas for growth, especially when it comes to women's health. Preventative care, reproductive health, and data surrounding these topics are all places where the country can improve.

Independent Research Review

⁵ Ibid, 37.

⁶ Iliriana Banjska. "Abortions in Kosovo." PrintingPress, 2019, <https://artpolis-ks.com/wp-content/uploads/2019/10/ABORTIONS-IN-KOSOVO-1.pdf>, 24.

⁷ Ibid, 29.

⁸ Dixit, Apoorva, et al. "Access to Healthcare in Kosovo." Rockefeller Center, 2016, https://rockefeller.dartmouth.edu/sites/rockefellerprod/files/kosovo_kwn_report_110416.pdf, 38.

I. Patient Outcomes

There are several important studies that have considered patient outcomes and experiences in Kosovo through different lenses. These studies have looked at rural and urban areas, majority and minority groups, etc. It is important to note that the majority of these studies do still focus on non-minority groups in Pristina, and there is less research surrounding ethnic minorities and their experiences in the healthcare system. Discussed below are several key studies which have contributed to the knowledge base regarding the experiences of patients in Kosovo.

One key study was conducted by Obas and her team. Obas and her team were concerned with the fact that Kosovo has the lowest life expectancy in the Balkans (at least, as of the start of the study in 2020)⁹. They conducted a study of 977 primary healthcare users, and assessed their general health as well as mental health indicators. Poor nutrition (85%), physical inactivity (70%), obesity (53%), and smoking (21%) were common risk factors found in ailing Kosovar residents.¹⁰ Depression was found to be a key factor as well, with 30 to 67% of primary healthcare users studying experiencing symptoms. Depression can lead to these high risk factors for ailment - depressed people are more likely to turn to overeating, drugs, alcohol, and smoking, etc. The study demonstrates an urgent need for stronger inclusion of mental health care in Kosovo, as well as better public health education, particularly for rural communities.

In an ethnically diverse country like Kosovo, it is important to consider the differences in patient outcomes for different populations. Luta and Dræbel focus on the experiences of Kosovo-Serbs in the healthcare system. They conducted semi-structured interviews of seven Kosovo-Serbs about their experiences with doctors, primary care, etc. The study found that Kosovo-Serbs have generally experienced an increase in the quality and accessibility of healthcare services since the war.¹¹ Longitudinal care and good experiences at consultations were key factors to patient satisfaction. However, Kosovo-Serbs still experience language barriers, as

⁹ Obas, Katrina Ann, et al. "Study Protocol: A Prospective Cohort on Non-Communicable Diseases among Primary Healthcare Users Living in Kosovo (KOSCO)." *BMJ Open*, vol. 10, no. 9, 2020, p. e038889, <https://doi.org/10.1136/bmjopen-2020-038889>.

¹⁰ Obas, Katrina Ann, et al. "Strengthening Primary Healthcare in Kosovo Requires Tailoring Primary, Secondary and Tertiary Prevention Interventions and Consideration of Mental Health." *Frontiers in Public Health*, vol. 10, 2022, p. 794309, <https://doi.org/10.3389/fpubh.2022.794309>.

¹¹ Luta, Xhyljeta, and Tania Dræbel. "Kosovo-Serbs' Experiences of Seeking Healthcare in a Post-Conflict and Ethnically Segregated Health System." *International Journal of Public Health*, vol. 58, no. 3, 2013, pp. 377–83, <https://doi.org/10.1007/s00038-012-0403-8>.

well as fear and anxiety about discrimination or dismissive attitudes based on their ethnicity. Some patients in the study noted particular discomfort when it came to seeing doctors for gynecological issues, and women's healthcare more broadly.¹² Continuing to build positive doctor-patient relationships eases these worries over time. It is important to note that this study was conducted in 2013, and inter-ethnic relations have certainly changed over the last decade. There is no newer research that exists on the topic, so it is unknown how exactly patient outcomes have changed for Kosovar-Serbs in the last decade.

An important element of healthcare is health literacy and education. Toçi, et al aimed to investigate the demographic and socio-economic correlates of functional health literacy among users of primary health care in Kosovo. Health literacy entails the capacity to obtain, process and understand basic health information needed to make appropriate health decisions. To determine the levels of healthcare literacy in Kosovo, they conducted a cross-sectional study of 1,035 healthcare users. Four out of five participants in the study showed inadequate or marginal health literacy.¹³ This is low in comparison to other Balkan countries, such as Serbia and Albania, and the authors suggest that increasing health-related education in schools would be a beneficial next step. This closely mirrors the conclusions of the larger institutional reports discussed earlier. A lack of healthcare related education in Kosovo is a common finding.

Coming back more specifically to women's healthcare, various studies have considered the level of care received by pregnant women in Kosovo. Kurti and Pelaj aimed to evaluate the knowledge of pregnant women in Kosovo about the importance of oral health to their pregnancy. 251 pregnant women were interviewed about their knowledge of the importance of oral health in regards to pregnancy. The study results indicate that 54% of the surveyed women know about oral health.¹⁴ Less than half, 27.9%, agreed on the importance of dental check-ups during the first trimester, and about 35.9% lacked knowledge that poor oral hygiene could cause premature birth.¹⁵ This shows a general lack of key healthcare knowledge among pregnant women in the municipalities of Kosovo.

¹² Ibid.

¹³ Toçi, E., et al. "Socio-Economic Correlates of Functional Health Literacy among Patients of Primary Health Care in Kosovo." *Public Health (London)*, vol. 128, no. 9, 2014, pp. 842–48, <https://doi.org/10.1016/j.puhe.2014.06.009>.

¹⁴ Kurti, Vlora, and Besarta Pelaj. "Evaluation of Knowledge and Oral Healthcare Practices among Pregnant Women in Pristina, Kosovo." *Revista Medicală Română*, vol. 71, no. 1, 2024, pp. 69–74, <https://doi.org/10.37897/RMJ.2024.1.11>.

¹⁵ Ibid.

Another study by Hoxha, et al aimed to investigate whether there was a relationship between the demographic factors of pregnant patients and their likelihood of getting a cesarean section in Kosovo. They surveyed 859 women who had low-risk pregnancies in public hospitals in Kosovo. It was found that women with monthly income and private health insurance had higher odds of getting a cesarean procedure. 20% of all low-risk births involved a cesarean section, which represents an overuse of this procedure, potentially due to doctors recommending the procedure more often than needed in order to increase profits¹⁶. Stronger healthcare education is recommended, as are corrective policies to ensure that cesarean sections are not recommended by healthcare providers (particularly in the private sector) when they are unnecessary.

From these studies, several trends become apparent. There is a lack of education related to healthcare in Kosovo, particularly when it comes to pregnancy and reproductive health. Health literacy, refers to a person's ability to access, understand, evaluate, and use health information to make the necessary medical decision for themselves. In Kosovo, this is generally low. Furthermore, different populations in Kosovo are experiencing the system differently, whether that be along gender or socioeconomic lines. For example, a man who is financially independent can make his own medical decisions due to him being the primary financer of such services. He would have the option to utilize health insurance and private medical services as he sees fit. However, a woman who is financially dependent on her husband might not necessarily be able to make her own decision regarding the type of services she can receive. This portrays how gender and economic status are interconnected factors that influence an individual's ability to access and utilize health services.

II. Hospital Workforce

In order to better understand how these patient outcomes come to be, it is important to analyze the hospital workforce conditions that generate them. Kelmendi, et al focus their research on the safety and support of healthcare workers. The researchers surveyed almost 200 healthcare professionals about Occupational Health and Safety Management institutions in Kosovo. The healthcare professionals explained several key barriers they face on a daily basis,

¹⁶ Hoxha, Ilir, et al. "Health System Factors and Caesarean Sections in Kosovo: A Cross-Sectional Study." *BMJ Open*, vol. 9, no. 4, 2019, pp. e026702–e026702, <https://doi.org/10.1136/bmjopen-2018-026702>.

including a lack of personal protective equipment, insufficient training, and weak enforcement of regulations.¹⁷ Based on the surveys, the authors concluded that while the legal frameworks in Kosovo are at EU levels when it comes to occupational health and safety, these frameworks are not always applied. Specific weaknesses to be worked on include updating equipment and improving human resources to decrease workplace injuries and occupational diseases experienced by healthcare workers.¹⁸

Primary healthcare is an essential part of any well-functioning healthcare system, and this study by Baraku and Pavleković aims to consider the effects of targeted training for quality coordinators. In Kosovo, “Quality Coordinators” oversee the evaluation of the primary healthcare system. Half of the Quality Coordinators were invited for training in Pristina, while the other half were not. The study found that the targeted training significantly increased Quality Coordinators’ knowledge of quality standards in two ways. First, the QCs were more capable of identifying previously overlooked standards; second, the organizational adherence to the standards also increased after the training.¹⁹ The level of improvement demonstrated a need for greater training of quality coordinators across the nation, which will eventually lead to greater quality of healthcare results due to better hygiene practices.

Including and developing all elements of the healthcare workforce is important to improving the system at large. Brajshori, et al aim to identify the knowledge and expectations regarding Advanced Nurse Practitioners (ANPs) in Kosovo's hospital environment. The authors conducted semi-structured interviews with thirty-two healthcare workers and asked them questions about their knowledge of the role of Advanced Nurse Practitioners. While an ANP track was established in 2021, it has not yet gained strong professional recognition according to the interviews: many health professionals in Kosovo lack knowledge of what an ANP does.²⁰ The study concluded that there needs to be greater awareness and education surrounding the great benefits ANPs bring to the medical community in order to diversify the healthcare workforce in the country.

¹⁷ Kelmendi, Albina, et al. “Challenges of Occupational Health and Safety Management in Healthcare Institutions in Kosovo.” *Materia Socio-Medica*, vol. 36, no. 3, 2024, pp. 149–54, <https://doi.org/10.5455/msm.2024.36.149-154>.

¹⁸ Ibid.

¹⁹ Ardita Baraku, and Gordana Pavleković. “Improving the Quality of Primary Healthcare in Kosovo: Impact of Targeted Training of Quality Coordinators.” *Medicina Clínica Práctica* 8, no. 1 (2025): 100479-.

²⁰ Brajshori, Naime, Petrit Beqiri, Vjosë Hajrullahu, Ali Ajeti, Ardita Ismajli, Astrit Behrami, Djellza Zenelaj, Kaltrina Zeka, and Klara Cahani. “Recognizing Attitudes and Expectations about the Role of Advanced Nurses (ANP’s) in Kosovo’s Healthcare System.” *E3S Web of Conferences* 585 (2024): 6003-. <https://doi.org/10.1051/e3sconf/202458506003>.

These studies demonstrate that there needs to be stronger support for medical professionals in the country. With a lack of professional development in support, the medical system suffers, particularly the public health sector. Improving training, oversight, and support for the medical professionals in the country is a necessary investment.

III. Healthcare System Management

In order for a healthcare system to properly function, it must be well managed. Afrim and Nadrire Loku explore how advancing information management systems can improve the healthcare system in Kosovo. The study employed a mixed-methods approach, combining both qualitative and quantitative research techniques. Based on interviews with hospital officials and the data hospitals provided, Loku found that better information management systems in hospitals in Kosovo reduced administrative costs, minimized redundant procedures, optimized billing and coding accuracy, and improved inventory management.²¹ The study recommended harnessing information management systems to improve professional training, information dissemination, and patient outcomes in Kosovo.

Management of the system itself is important, and so is the way it is funded. Elshani and Leke investigate the effectiveness of public health and education expenditures in Kosovo and compare these results to the rest of the Western Balkans. According to their metrics, Kosovo ranks 39th out of 42 countries in their sample when it comes to healthcare sector performance. According to their economic analysis, Kosovo is not particularly efficient when it comes to public health expenditures.²² The authors recommend revisiting public health spending patterns to increase efficiency and create better patient outcomes.

It is also important to consider what stakeholders are included in the conversations surrounding healthcare system management. Hoxha et al were concerned with how unequal power distribution in the Kosovar societal system may impact healthcare access. Based on this, they investigated the effects of social participation of vulnerable groups in the healthcare system.

²¹ Loku, Afrim, and Nadire Loku. "Management Optimizing Healthcare Delivery through Advanced Information System Management: A Financial and Operational Perspective for Kosovo's Healthcare Sector." *Pakistan Journal of Life and Social Sciences* 22, no. 2 (2024). <https://doi.org/10.57239/PJLSS-2024-22.2.0063>.

²² Elshani, Alban, and Lekë Pula. "The Efficiency of Educational and Healthcare Public Expenditures in Kosovo: A Data Envelopment Analysis Technique." *Ekonomika - Vilniaus Universitetas*, vol. 101, no. 2, 2022, pp. 92–108, <https://doi.org/10.15388/Ekon.2022.101.2.6>.

Specifically, the study focused on ethnic minorities of Roma, Ashkali, and Egyptian backgrounds, elderly citizens, and people with disabilities. To study this, the researchers utilized patient councils, community scorecards, and municipal health action plans to increase social inclusiveness. It was found that these mechanisms do create stronger relationships between patients and providers that would be beneficial to minority patients.²³

These studies show a general inefficiency in the healthcare system. While these studies do not specifically address women's healthcare issues, it should be obvious that if a system is inefficient in general, that affects women who the system is meant to serve as well. Some insufficiencies include:

- Lack of protective equipment for healthcare workers
- Insufficient medical training for healthcare workers
- Weak enforcement of important medical regulations that ensure health and safety of both patients and providers
- Lack of knowledge on what the professional obligations are for certain medical workers
- Lack of public campaigns that advertise medical services, specifically for those who live in rural areas

There is room to grow when it comes to the financing, information management, and inclusion of stakeholders in the healthcare system in Kosovo.

Kosovo Legal Framework on Healthcare

The legal framework surrounding women's access and use of healthcare services within the country rely primarily on the following documents: **Law on Health** (LAW No. 04/L-125 ON HEALTH) adopted in May 2013 that aims to establish legal grounds for the protection and the improvement of the healthcare for citizens of the Republic of Kosovo through health promotion, preventive actions and provision of comprehensive and quality healthcare services. In chapter 1, paragraph 3 of article 4 emphasizes the right for all citizens to have equal access to healthcare

²³ Hoxha, Ilir, Vlora Basha-Berisha, Erza Selmani, Shege Bahtiri, Ariana Bytyci-Katanolli, Ardian Imeti, Paula Valentine, and Jana Gerold. "Social Participation in Health – Case Studies from Primary Health Care in Kosovo." *BMC Public Health* 24, no. 1 (2024). <https://doi.org/10.1186/s12889-024-21053-z>.

services. In chapter 2, paragraph 1.2, article 5 states that: *“Inclusiveness and non-discrimination: equal healthcare for all citizens and residents by ensuring the standards during fulfilling the needs at all levels of healthcare as well as ensuring healthcare without discrimination on basis of: gender, nation, race, color, language, religion, political preferences, social status, sexual orientation, the level of physical or mental abilities, family status, or age”*²⁴. Articles 6 and 7 state that healthcare institutions are legally required to implement the healthcare principles and are subject to supervision by the Ministry of Health. Article 12 emphasizes the prevention and early detection of diseases such as cancer, addiction, STDs and HIV. Additionally, it supports services that aid women during pregnancy and postpartum, and ensures the healthy development of children. Chapters on financing healthcare, guidelines on termination of pregnancy and list of healthcare professionals are included in law and contribute to the overall healthcare infrastructure.

However, the law fails to mention the implementation of a “Health Information System”, the promised digital infrastructure that would standardize and organize consistent data collection and retainment for citizens.

Article 49 Paragraphs 1 and 2 outline the “Health Integration System” as follows: “The Ministry ensures establishment, development, and functioning of the unique and integrated health information system (hereinafter as HIS) .in the whole health sector, regardless of the type of healthcare institution, type of ownership, or organization, based on the normative’s and standards set by the Ministry...All healthcare institutions, responsible physical and juridical persons are obliged to collect health data and report them in the HIS, in compliance with the sub-legal act issued by the Ministry.” The following articles of Chapter 9 specify organization of the HIS, methods of data collection and reporting, ownership and responsibilities of the data, data access, and supervision of the HIS.

Additionally, gynecology services are not even mentioned throughout the entire document as a fundamental need for women. This law provides a framework that supports women’s access to healthcare through non-discrimination, maternal care and preventative disease services. However, it fails to address women’s healthcare needs on a full spectrum.

²⁴Assembly of the Republic of Kosovo, *Law No. 04/L-125 on Health*. Official Gazette (“Gazeta Zyrtare”), 7 May 2013, https://cps.rks-gov.net/wp-content/uploads/2020/08/LAW_No._04_L-125_ON_HEALTH.pdf.

The Law on Health Insurance²⁵ (LAW NO. 04/L-249), was passed in 2014 and ensures that citizens of the Republic of Kosovo have access to universal healthcare services to address medical service needs and financial protection against impoverishment. Kosovo is the only country in the Western Balkans that does not have established public health insurance for citizens. Those who cannot afford private insurance are put at a disadvantage, having to depend on the public health system, which has been found to be poor in quality and medical services²⁶. Mandatory health insurance would be required for all citizens and be solely provided by the national Health Insurance Fund (Art. 6). Guaranteed services would include emergency care, basic services for children under the age of 18 and essential care for pregnant and postpartum women (Art. 7). This law put an emphasis on equal access to health insurance (Art. 16). Exclusions for services include experimental treatments, clinical trial medications, cosmetic/reconstruction surgery and fertility assistance. This law makes important strides in ensuring women's access to essential health services by mandating care for pregnant women and establishing universal healthcare. Gender equity is upheld through this law by entitling women, like all citizens, access to health care regardless of income, social status or place of residence. Services for both mothers and children are recognized and support a need for reproductive and material healthcare. However, limitations in the law portray gaps in essential care for women's overall health. Services like contraception, abortion care, gynecological services and preventative female cancer services are not explicitly mentioned. In addition to this, there comes a financial burden for uninsured women who fail to enroll in mandatory insurance due. This could be due to lack of information regarding coverage, legal status or ability to make their own decisions in the patriarchal structure of home life. This law was passed by the Assembly, but has yet to be fully implemented.

LAW NO. 08/L-044 ON AMENDING AND SUPPLEMENTING LAW NO. 03/L-110 ON TERMINATION OF PREGNANCY, was proposed to amend and supplement the existing Law No. 03/L-110 on Termination of Pregnancy, in order to align it with Law No. 05/L-087 on Minor

²⁵Assembly of the Republic of Kosovo. *Law No. 04/L-249 on Health Insurance*. Official Gazette ("Gazeta Zyrtare"), 30 Apr. 2014,

https://cps.rks-gov.net/wp-content/uploads/2020/09/LAW_NO_04_L-249_ON_HEALTH_INSURANCE.pdf.

²⁶Paçarizi, Gentiana. "Health Insurance: What, Why and When?" *Kosovo 2.0*, 4 Sept. 2024, <https://kosovotwopointzero.com/en/health-insurance-what-why-and-when/>.

Offences. The already established Law for Terminating of Pregnancy establishes the legal basis for the regulation and implementation of pregnancy terminations in Kosovo. Specifically, it states the protection measures to limit the risk of women who decided to have the procedure done illegally. This law goes one step further to outline the fines and penalties for legal violations by health institutions and individuals during termination of pregnancy services. As stated, fines ranging from €500 to €20,000 will be instated if abortion is in violation of certain established Articles 4, 5, 15, or 16 of the law; advertising illegitimate abortion services; failing to report abortion statistics. These amended articles hold health institutions and medical professionals accountable for complying with abortion laws. This ensures that unlawful or unregulated practices of abortion services are penalized and that women's health and safety are upheld. It also offers the opportunity for accurate data collection on women's reproductive health and maternal services to better monitor trends in health statistics.

Administrative Instruction No. 04/2020 on Primary Health Care²⁷ issued by the Ministry of Health of the Republic of Kosovo, defines how primary health care (PHC) institutions in Kosovo are organized, via operations, infrastructure and staffing. PHC offers the ability for citizens to have access to health services, such as education, prevention and emergency care. As outlined in health services provided, during the healthcare workers are obliged to women victims of sexual violence should be provided with emergency contraception. Additionally, they should be referred to an obstetrician-gynecologist and case management. Section 6 refers to the health services and reproductive healthcare that include prenatal/antenatal care; counseling services; nutrition education; ultrasonographic examinations and screenings; menopausal care; etc. This instruction promotes lifelong support through reproductive, maternal and aging stages of a woman's life. It also extends services to rural/remote areas through the establishment of Family Medical Ambulance (FMAs). The structure and scope of this administration instruction incorporates women's access to services and preventative measures to best assist contribute to a functioning primary care environment.

²⁷Ministry of Health of Kosovo. *Administrative Instruction Primary Health Care*. Ministry of Health of Kosovo, 2020, <https://msh.rks-gov.net/Documents/DownloadDocument?fileName=UA%2004-202046929520.6693.pdf>.

Fieldwork Findings

The following section provides the results of our study that was conducted by the Jahjaga Foundation. The previous sections have offered contextual background to Kosovo's healthcare infrastructure. This portion focuses on key findings taken from our fieldwork. A combination of interviews with NGOs and Google Form surveys that were administered in various municipalities across Kosovo help to formulate the data that you will see portrayed in the graphs. Our research team has analyzed all findings into four main areas that impact women's access to healthcare: (1) proximity and geographic access; (2) affordability and financial barriers; (3) cultural and social factors; (4) systematic and capacity limitations.

1.) Proximity and Geographic Access

Proximity to health facilities is a major barrier for both men and women living in more rural areas. Urban areas have stronger health infrastructure, with rural areas facing compounded access issues from staffing, distance, and transport. "Big cities have more specialist doctors, laboratories and private clinics. In small municipalities and mountainous areas, the lack of public transport, the distance from the nearest hospital, and the lack of staff makes access difficult," (PRAK). People from rural areas in Kosovo are interested in getting checkups and are aware of their healthcare needs, but willingness to seek care is undercut by their physical distance away from healthcare facilities (Jeta Vita). This affects women to an additional degree because female-specific healthcare, such as screenings of breast/cervical cancer, are generally only conducted at gynecological practices with specialist equipment, labs, and doctors. "Residents of rural and remote areas generally have more limited access to specialized health services, diagnostic equipment, and qualified medical professionals...Mammography and Pap smear services are mainly concentrated in urban centers, and there is still no fully functioning national screening programme for either cancer or for other conditions, " (UN). Figure 1 shows the locations of all health practices, including clinics, hospitals, dentists, and laboratories throughout Kosovo. Gynecology offices that regularly provide the aforementioned services are concentrated in the major cities, namely Prishtinë, and sparse throughout the rest of the country.

Women's Health Facilities in Kosovo by Population Density

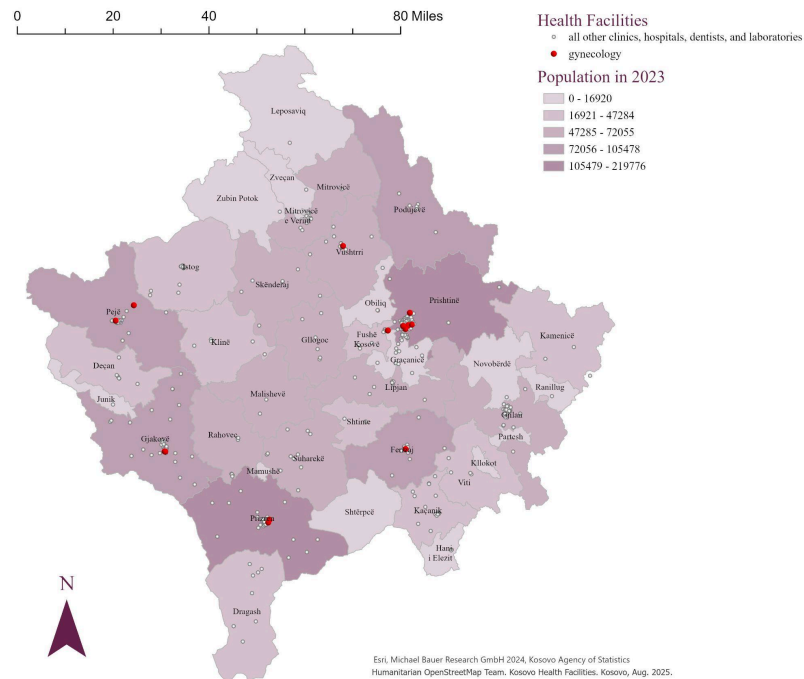


Figure 1

In an effort to combat this access barrier, Jeta Vita successfully advocated for a mobile mammograph unit with the support of President Jahjaga, which can partially offset geographic disparities if supported with infrastructure and other advocacy. It is still in use and can be used in family centers for rural women who are not in close proximity to a gynecology center (Jeta Vita).

2.) Affordability and Financial Barriers

Underfunding severely limits both basic and specialized healthcare in Kosovo, resulting in inconsistent access and overburdened services for medical professionals. Across a majority of NGOs interviewed, there was a common theme mentioned on the widespread lack of health insurance and ongoing medicine shortages. This made even basic medical care financially and logistically inaccessible. The issue became even worse when marginalized groups were involved when needing such services. UNFPA stated that “healthcare expenditure in the budget was the lowest in the region.” This does not even mention the specialized services that are oftentimes called for as risk patients. Medical specialties such as oncology, radiology and mastology, call for specialized doctors and medical treatments. Jeta Vita added that there are only 15 mammography machines available in all public institutions, noting that the private sector is much more equipped when compared to the public sector. Therefore, the healthcare budget limits patients’ ability to have these crucial services accessible for people. PRAK notes that the lack of insurance and lack of overall access to life-saving medicine is a deep-seated systematic problem that affects all citizens, men and women, and a much more significant impact to the most marginalized groups. Most patients rely on out-of-pocket payments, creating significant barriers to preventative care and timely treatment. UNFPA states that many do not have the means to pay for private healthcare insurance. The absence of coordination between healthcare levels further delays diagnoses and care. Women, facing greater economic vulnerability, are disproportionately affected by these systemic issues. While diagnostic services like mammography are more available in the private sector, financial constraints prevent equitable access. Women’s lower economic independence directly reduces their ability to seek timely and preventative healthcare. PRAK stated that “women are more economically vulnerable and the system fails to address these barriers.”

Worried about not being able to pay for medical costs in case of a serious illness or accident
(% age 15+)

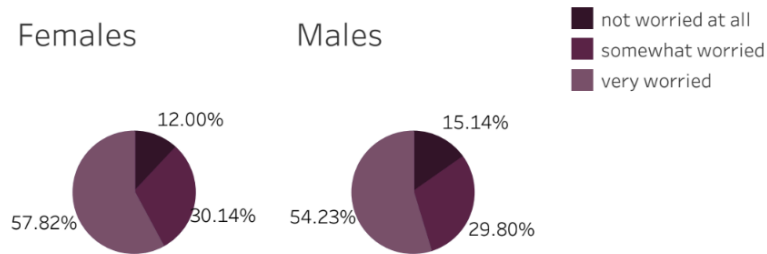


Figure 2

Accessibility Factors that Patients Say Need Improvement

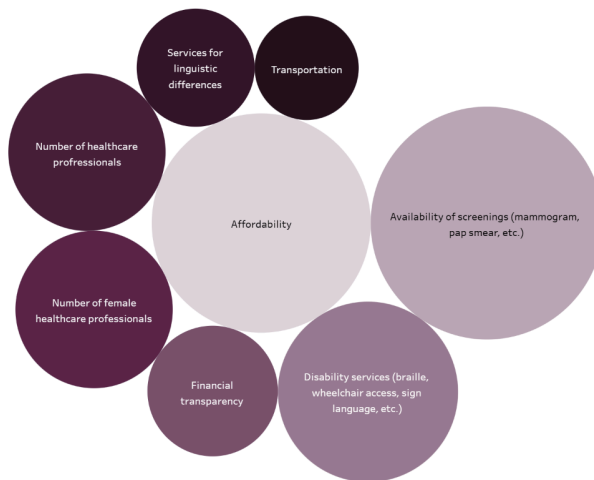


Figure 3

Bubble size indicated the proportion of patients who identified this accessibility factor as a barrier needing improvement.

In a study conducted by the Jahjaga Foundation, more than half of men and women surveyed reported being ‘very worried’ about not being able to pay for medical expenses when illnesses or an accident happens. In addition to this, affordability and availability of screenings, such as for a mammogram or pap smears posed a significant barrier for women. One disagreement amongst the data showed that healthcare professionals didn’t indicate that there was a problem with affordability in healthcare. This portrays a discrepancy when compared to patient feedback

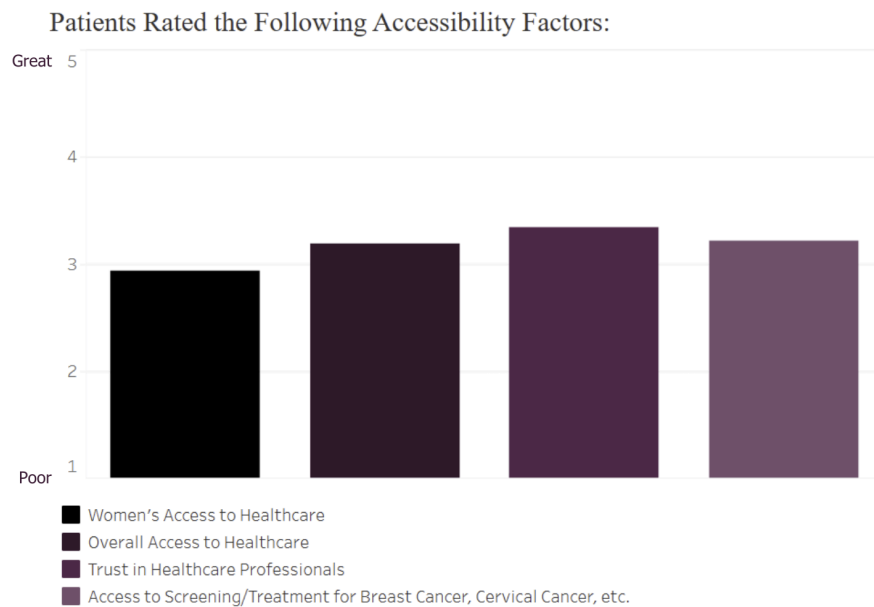
stating that finances were a large obstacle in access to medical services. Here reveals gaps in perception between healthcare professionals and patients, which have consequences for the quality and accessibility of necessary care for women.

3.) Cultural and Social Factors

Cultural and social dynamics also continue to shape women's access to healthcare in Kosovo, with gender roles, stigma, and cultural preferences affecting how and when women seek care. This can particularly be seen among women in traditional care-giving roles, who consistently put family needs ahead of their own and often postpone or neglect check-ups and treatments. This is reinforced by the unequal distribution of domestic activities, leaving women in Kosovo with little time or energy to prioritize their own health (Jeta/Vita & PRAK). Social stigma around reproductive and mental health further adds to these delays, making care-seeking seem shameful or inappropriate (PRAK). Therefore, NGOs such as Jeta Vita have claimed that grassroots advocacy is necessary to make these conversations more acceptable in public and clinical spaces (Jeta Vita).

These cultural pressures aren't just internal, as they're externally evident within structural gaps in the health system. Although awareness of preventative care is perceived by the interviewed NGOs to be relatively high, the services needed to act upon this, such as screening equipment or specialized providers, are often out of reach (Jeta/Vita). Additionally, general trust in healthcare professionals and perception of the accessibility of the healthcare system appears to be mediocre at best, which would further limit women's motivation to seek medical care (Figure 4).

Figure 4



Being treated by female medical professionals was not recognized as the largest barrier to women's access to healthcare (Figure 3). Similarly, a significant portion of patients reported that they would prefer to be seen by a female doctor, though not the majority (Figure 5). However, NGOs working in women's healthcare in Kosovo "observe that women—particularly mothers and young girls—feel safer and more at ease when receiving healthcare services from a female provider," (AMC). Recognizing this, some organizations such as the Action for Mothers and Children integrate female medical providers into maternal and childcare programs. They also try to deliver care in welcoming, non-judgemental environments and to build relationships that encourage women to return for follow-up care (AMC). By addressing these cultural and social factors, initiatives such as that one can help bridge the gap between awareness and actual medical service usage by women.

Do Female Patients Prefer to be Seen by Female Doctors?

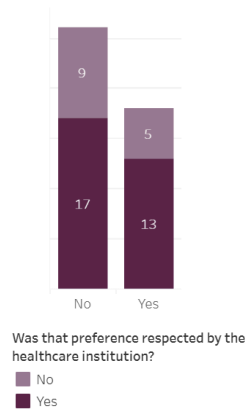


Figure 5

Comfort is a particularly important factor when patients need to discuss more taboo topics related to sexual and reproductive health. Women are consistently less well informed on these topics than their male counterparts (Figure 6), and usage of modern contraceptive methods is not widespread (Figure 7). Having consistent access to female healthcare professionals, education, and gynecological care is the key to bringing equity to women's healthcare in Kosovo. The medical professionals interviewed unanimously agreed that their practices had female staff members, and 50% stated that female health services were simply underutilized and undersought by Kosovo women. This highlights the importance of outreach programs like that of the AMC to fight the cultural stigmas that prevent women from taking advantage of the services available.

Comprehensive correct knowledge of HIV/AIDS, ages 15-49, (2 prevent ways and reject 3 misconceptions)

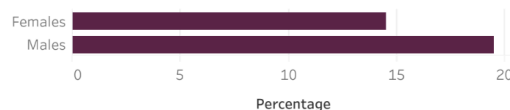


Figure 6

Knowledge of HIV, is the percentage of respondents who correctly identify the two major ways of preventing the sexual transmission of HIV (using condoms and limiting sex to one faithful, uninfected partner), who reject the two most common local misconceptions about HIV transmission, and who know that a healthy-looking person can have HIV.



Figure 7

Contraceptive prevalence, any method is the percentage of married women ages 15-49 who are practicing, or whose sexual partners are practicing, any method of contraception (modern or traditional). Traditional methods of contraception include rhythm (e.g., fertility awareness based methods, periodic abstinence), withdrawal and other traditional methods.

Contraceptive prevalence, any modern method is the percentage of married women ages 15-49 who are practicing, or whose sexual partners are practicing, at least one modern method of contraception. Modern methods of contraception include female and male sterilization, oral hormonal pills, the intra-uterine device (IUD), the male condom, injectables, the implant (including Norplant), vaginal barrier methods, the female condom and emergency contraception.

Unmet need for contraception measures the capacity women have in achieving their desired family size and birth spacing. Many couples in developing countries want to limit or postpone childbearing but are not using effective contraception. These couples have an unmet need for contraception. Common reasons are lack of knowledge about contraceptive methods and concerns about possible side effects.

4.) Systemic and Capacity Limitations

Kosovo's healthcare system is constrained by geographic barriers, affordability issues, and cultural factors, all of which limit access to essential medical care, particularly for women, further inhibiting their ability to maintain overall health and well-being. Human resource shortages and inadequate equipment further constrain both preventive and treatment efforts (Jeta Vita). Preventative care is poorly integrated, especially in rural areas, and the absence of an

organized national screening program or post-diagnosis support network hinders early detection and structured care (PRAK). The UNFPA reports on weak health information systems and lack of useful technology prevent efficient coordination between professionals and patients. Seasonal campaigns, although useful, lead to compressed demand and missed opportunities for preventative care. This past year, the Ministry of Health promoted breast cancer awareness during the month of October. Though great for the overall awareness of the disease, it has been observed that people only think to do check-ups in October (Jeta Vita). Awareness for preventative care needs to exist year-round. NGOs frequently fill funding gaps by donating essential medicines and equipment. While these supplemental ways of funding are necessary, it further demonstrates the public healthcare budget's inadequacies in financial help.

Data Analysis and Conclusion:

Key insights gathered from women's surveys:

- Women gave the healthcare system and their level of access an average rating of 3 out of 5.
- Doctor visits generally occur 1–2 times annually for most individuals.
- Respondents gave a trust rating of 3 out of 5 for the healthcare system.
- Most respondents indicated they have never had a preventive care visit.

Key insights gathered from healthcare providers surveys:

- Generally, workers seem to state that women have easy access to the healthcare system.
- Hospitals need to improve their services for people with disabilities.
- The average wait time for preventive services is approximately one week.
- There are female workers at women's disposal.

Key insights gathered from NGO interviews:

- Healthcare underfunding limits the capacity and accessibility for patients
- Lack of insurance lowers women's economic and health independence

- Cultural expectations such as caregiving often lead women to prioritize family obligations rather than their own health and well being
- Although rural populations generally have access to health care services, physical distance from main health facilities limits their access to utilizing such services
- An integrated health information system and long-term campaign are needed to promote care coordination across all networks and provide year-round care

Based on the data collection, surveys, and interviews conducted, it is clear that Kosovo is not completely lacking in the technology or expertise to meet the medical needs of its population, nor is the population severely lacking interest or understanding about medical issues. However, there are several factors that limit accessibility for any non-general medical issue cared for by the public system. The female population of Kosovo is one of the main groups to be impacted by this with regards to female-specific medical needs. Most gynecological practices are privately run, making them more unaffordable. There are also not many of them, with the majority concentrated in just the major cities. These two factors alone make it much more logistically difficult for the average Kosovar woman to care for her health. Furthermore, there is a prevalent cultural perspective that a woman's health is not a top priority, and social stigma against seeking sexual and reproductive services. These cultural factors are not uncommon globally, including in countries with more developed medical infrastructure. However, social stigma is all the more impactful when coupled with logistical barriers. Much of the advocacy for women's screenings, as well as the technology and professionals to conduct them, are provided by NGOs, despite Kosovo's legal responsibility to provide equitable medical care to its people. These persistent issues have largely gone unaddressed by Kosovo due to another failed infrastructure promise: the development of the Health Information System. The HIS, which would maintain digital records of medical history for all patients, would allow for long term data analysis by the Ministry of Health, which could then make a more accurate and specific determination of which populations are lacking access to which medical practices.

Recommendations:

Improve the geographic distribution of healthcare providers and diagnostic equipment

There should be established nationwide screening programmes, such as for breast and cervical cancer, that provide public healthcare providers the resources to serve communities throughout the country, even in the most rural areas.

Increase public health and/or municipal budgeting through prioritizing health insurance and primary/preventive care

Allocating necessary funding for the healthcare system is a crucial component of institutional success. There should be an established fund, as in the one stated in the Law on Health Insurance though never implemented, to ensure proper financial areas of medical care can be covered for all citizens no matter their socio-economic or geographic status.

Implement structured education systems and outreach programs that expand awareness and fight against social stigmas surrounding women's health

Building a strong and long-lasting healthcare system starts with an educated and knowledgeable community regarding women's healthcare and needs. Comprehensive youth programs that promote sexual, reproductive and preventive diseases protocol within school systems will de-stigmatize the subject as a "taboo" topic. Additionally, education for healthcare workers and other medical professionals can be implemented to provide continuous training with a focus on gender sensitivity and inclusion.

Strengthen health information systems and data collection to better inform policy

The national Healthcare Integration System should be established and implemented for all Kosovar citizens. Resources like hardware and data training programs should be provided for medical professionals in order to collect data on demographics, geographic location, and utilized health services to determine which communities of women are underserved in cancer testing

and treatment, reproductive and sexual health education, general medical care access etc. Additionally, there needs to be enhancements to inter-institutional coordination and integrated services into national systems to promote sustainability. This will be more easily introduced with the backing of a unified HIS which will allow legislative and executive decisions on public health to be more closely aligned with the medical needs of the people and the capabilities of the supporting NGOS. Additionally, communication networks could be established for different village counsels to share information regarding patients and medical services.